MercyMed Website PRD Template

Team 1 MercyMed PRD

Make ONE Google Doc copy per team. Name it “Team 1 MercyMed PRD.” Add members as editors and share the team copy with fuming@gmail.com. Everyone contributes; the coordinator tracks time and input. Agree on a concise working version in the first 35 minutes. Each student later uses the same team PRD for an individual AI-built site.

Team: [1] Members: [3] Date: [28] Version: [1]

Labels: CONFIRMED CLIENT INPUT = supported by the client sources below. TEAM DECISION = the way the team chooses to build the website, with a reason. ASSUMPTION = an idea that is not yet confirmed. OPEN QUESTION = missing information or a decision still needed.

Source key: Q = Website Client Questionnaire - MercyMed (Oct 2026), 2 pages. P = Dr Lukas’s MercyMed presentation, 19 PDF pages. Example: Q p2 means questionnaire, page 2.

1. Project Goal and Success

Client problem: The client wants a business website for their clinic

Website goal: is to get known wider, to get online access for customers easier and get more people (locals and tourist)

Visible success: Fast, easy, friendly interface and solve the problem

2. Users and Main Tasks

Primary user and need: [User: Local Bishkek residents, international expats, and tourists/travelers seeking family or pediatric medical care.] [Need: An easily navigable, multilingual web platform to review clinic services and doctor credentials in their language, and easily request an appointment.] [Evidence: *Website Client Questionnaire* p. 1 (items 1 & 6) and *Dr. Lukas's Presentation* pp. 9–11.]

Priority task 1: When a visitor wants to find information about the clinic and its staff, what their clinic offers they will see that information in the website.

Priority task 2: [When a visitor wants to book an appointment, they can click the prominent "Book Appointment" button, fill in a short form with their basic details (Name, DOB, Phone Number), and submit a pre-filled WhatsApp message directly to the clinic receptionist.

Evidence reference: [Source/page: *Website Client Questionnaire* p. 1 (item 1 & 6), p. 2 (item 11); *Dr. Lukas's Presentation* pp. 9, 15–16; *Transcript* [00:17:00], [00:55:16].]
[What it supports:Supports that bringing more patients through discoverability and frictionless appointment requests is the clinic's primary goal, and confirms WhatsApp is the client's preferred workflow for booking.]

Client context. Your team still chooses the main user: local residents, tourists and expats, plus institutional and training partners (Q p1; P pp9–10). Older and middle-aged users and low-to-medium tech comfort are client expectations, not validated research (Q pp1–2; P pp10–11).

3. Scope and What We Will Not Build

Scope for this version: A responsive prototype covering the core essential pages/sections: Home (hero banner, mission tagline, WhatsApp booking CTA), About Us (evidence-based medicine philosophy and clinic history), Medical Services (consultations, diagnostics, ultrasound, labs overview), Team (staff roster with 2-sentence bios and specialties, no separate sub-pages), Mobile Clinics / Charitable Work (community outreach), and Contact (location near Ayu Grand in Tunguch, map embed, direct phone and WhatsApp links).

Main call to action (CTA): [Action: "Book Appointment" / "Book Consultation" (initiating a WhatsApp chat with patient name, DOB, and phone number).

[Location: Fixed prominently in the top navigation header and within the homepage hero banner.]

[Reason: Fulfills the clinic's #1 objective to convert web traffic into scheduled clinic visits by removing booking friction.]

What we will not build now: [Item: Live two-way OpenEMR database scheduling integration, patient portal for viewing lab results, and direct online payment processing.]

[Reason or dependency: Direct OpenEMR integration introduces double-booking risks and server complexity beyond an initial prototype scope; lab results are handled externally by Express Plus via QR codes; direct payment is not required for initial consultation booking.]

Shared across team versions: [Light background theme, yellow-orange and grey brand palette, real photographs (no AI images), clean non-Arial modern typography, three-language navigation support (English, Kyrgyz, Russian), and WhatsApp-based booking simulation.] Home, About, Medical Services, Corporate Clients, Team, Medical Education, Mobile Clinics or Charitable Work, and Contact (Q p1; P p15). P p15 also asks about International Patients and online booking. This is not a fixed page count for the first version. Agree on a realistic scope. Ask the instructor if the scope is unclear.

4. Requirements and Priorities

Write 4–6 clear requirements. A requirement says what the website must do. Use Must, Should or Could for priority. An acceptance check is a simple test: write the action and expected result.
WORKED EXAMPLE — REQUIREMENT CONFIRMED BY A CLIENT SOURCE + SAMPLE PRIORITY/TEST
ID R0. Source-confirmed requirement: The website offers English, Kyrgyz and Russian (Q p2; P p11). Illustrative priority: Must. Illustrative test to adapt: switch language on two sample pages; navigation and selected language remain consistent, and text awaiting review is visibly labeled.
R1 [Priority Must] [Requirement The website must support English, Kyrgyz, and Russian.] [Source Q p2; P p11] [Test Switch between languages → navigation and available content display correctly.]
R2 [Priority Must] [Requirement Visitors must be able to find medical services and contact information.] [Source Q p1; P p15] [Test Open Medical Services and Contact pages → relevant information is visible and easy to find.]
R3 [Priority Must] [Requirement The website must work on desktop and mobile devices.] [Source TEAM DECISION] [Test Open the website on desktop and mobile → navigation, text, and buttons work correctly.]
R4 [Priority Must] [Requirement The website must use a professional, simple design with a light background and the client's brand colors.] [Source Q pp1–2; P p13] [Test Open the main pages → colors, typography, and spacing are consistent.]
R5 if needed [Priority Should] [Requirement Visitors should be able to access information about MercyMed, its medical team, and relevant programs.] [Source Q p1; P p15] [Test Navigate to the relevant pages → information is clearly presented.]

5. Content Design and Languages

Essential content and approved files: [MercyMed logo, medical services, team information, contact details, and real photographs if approved.] [Source/owner: Dr. Lukas / MercyMed; publication approval to be confirmed.]
CONFIRMED CLIENT INPUT: professional, informative and uncluttered; light background; logo yellow-orange and grey; real photographs, not AI-generated images; website languages English, Kyrgyz and Russian (Q pp1–2; P pp11,13). The client disliked Arial in website references. Treat this as a website typography preference to respect or clarify, not a lecture-slide font rule.
Team choices: [Typography: Open Sans, avoiding Arial.] [Hierarchy/spacing: Clear headings, short paragraphs, readable text, and enough whitespace.] [How choices follow client direction: Professional and uncluttered design, light background, yellow-orange and grey colors.]

Navigation: [Labels: Home, About, Medical Services, Team, Contact.] [Persistent navigation/way back: Header navigation on every page and a clickable logo returning to Home.] [Mobile behavior: Collapsible navigation menu.]
Real photos: [Source: MercyMed / client-provided photos.] [Permission status: Not yet confirmed.] [Fallback if unresolved: Use simple icons or neutral design elements.] Do not fabricate staff, patients or facilities. Permission remains open, especially for children or clinic attendees.
Languages: [Coverage: English, Kyrgyz, Russian.] [Reviewer: Client-approved reviewer, to be confirmed.] [How unreviewed translation is labeled: Translation pending review.]
Missing content: use "[Opening hours: awaiting client confirmation]" or omit the claim. Never invent services, credentials, testimonials, contacts, hours or prices.
Future updates: [What should be easy to update? Medical services, team profiles, contact information, and announcements.] [Who may update it? Authorized MercyMed staff; CMS not yet selected.] CMS choice is not specified in the supplied sources (Q p2; P p16).

6. Quality and Safety Checks

Recommended classroom checks, not certification.
Mobile task — Target [Visitors can navigate the website and find contact information on mobile.] [Check Open at 375px width → navigation and contact information are accessible.]
Keyboard use — Target: each control works with a keyboard and shows a visible focus mark. Check [Use Tab, Shift+Tab, and Enter → all interactive elements can be accessed without a mouse.]
Contrast/alt text — Target: ordinary text 4.5:1; large text 3:1; meaningful images have useful alt text. Check [Test text contrast and inspect image descriptions → requirements are met.]
Mobile reading — Target: normal text fits the screen without horizontal scrolling. Check [Open at 320px and 375px widths → text remains readable without horizontal scrolling.]
Speed test context — Target [Home page / mobile device / simulated mobile network.] [Check Run Chrome Lighthouse → record actual performance results and identify issues.]
Privacy/prototype boundary — INSTRUCTOR GUIDANCE: no real patient data, live patient-data collection, real booking/payment submission or OpenEMR access. Mark any booking simulation as a prototype. Check [Verify no forms submit real patient information and any booking demonstration is labeled "Prototype only."]

Easy updates — Target [Service descriptions, contact details, and team information can be changed easily.] [Check Update sample content → changes appear correctly without breaking the layout.]

7. Assumptions and Open Questions

#	Type	Topic	Impact	Evidence / person needed	Temporary choice

1	OPEN QUESTION + RISK	Booking	Booking is exploratory. Two ideas were discussed: a form that sends a WhatsApp message to th e receptionist, or an OpenEMR integration. Neither is approved, and no endpoints, fields or rules are given (Q p2; P pp16, 18). This decides scope and timeline. An OpenEMR integration would also handle patient data, which brings privacy and security risk.	Clinic director to choose the approach. OpenEMR admin to confirm version, API access and calendar rules. Receptionist' s WhatsApp number.	Reversible decision: a "Request an appointment" button that opens WhatsApp with a pre-filled message. No OpenEMR integration in v1. Number shown as [WhatsApp number TBC] until confirmed.

2	OPEN QUESTION	Instagram	An Instagram feed is listed (P p16), but its priority is open (Q p2). No account handle is supplied. A live feed also needs ongoing API access and token upkeep.	Client to confirm the account handle and whether the feed is required or nice-to-have.	Placeholder: static "Follow us on Instagram" link with [handle TBC]. Add a live feed later if confirmed.
3	ASSUMPTION + RISK	Contacts / location	"Tunguch microdistrict, near Ayu Grand" (P p4) is approximate, not a verified street address; a wrong map pin sends patients to the wrong place. No verified clinic phone or email is supplied. The telephone on Q p1 belongs to the logo designer and must be excluded.	Clinic director to give the exact address, a map link, and the clinic phone and email.	Placeholder: show "Tunguch microdistrict, near Ayu Grand, Bishkek" with no map pin, plus [Phone TBC] and [Email TBC]. Never use the Q p1 number.

8. Team Review and Next Steps

Team-agreed version

Version: v1.0 Date: 30/10/2026

Member reviews

Name	Date reviewed
Atai Sultanbekov	30.10.2026
Ismailov Isroil	10.30.2026
Elizaveta Alpatskaia	30.10.2026

Change log

Version	Date	Change	Reason
v1.0	30.10.2026	First team-agreed version of the PRD	Team review completed

Individual next steps

Student name	AI tool	Project link	Next step
Atai Sultanbekov	Claude code/Claude Design	TBC	Create the main page of the website
Ismailov Isroil	Stitch, AI studio, Claude, UIverce	TBC	Create at least demo working website
Elizaveta Alpatskaia	Lovable / Claude	TBC	Create a responsive MercyMed website prototype.